

Diabetes mellitus and insulin use in adult social care

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Categories: Organisations we regulate

Insulin is a hormone that helps to regulate metabolism and lower blood glucose levels.

It is most often prescribed for patients with [type 1 diabetes](#). It is sometimes also used to treat [type 2 diabetes](#) when other methods have been unable to control blood glucose levels.

Many different insulin preparations are available in the UK. There are 3 main groups of insulins:

- fast-acting
- intermediate-acting
- long-acting.

Insulin is usually given by sub-cutaneous injection. There are many devices available for administering insulin. These are often designed to make it easier for patients and carers to administer.

Care planning

You should have a person-centred care plan for anyone using insulin. This should be reviewed regularly and when people's needs change.

This should include:

- an assessment of the support a person may need to manage their diabetes care, including who will administer insulin
- clear information on the person responsible for all aspects of care
- the details of the person(s) responsible for providing any specialist support
- information on the device and the correct equipment needed
- information to support safe administration of insulin, for example rotation of administration site, and safe use and disposal of device and equipment
- information about blood glucose level monitoring, including the frequency and acceptable range and who will carry out this monitoring
- what to do if blood glucose levels are outside of the acceptable range
- the signs and symptoms for low or high blood glucose levels and the appropriate treatment
- when to seek support from the GP, community nurses, diabetes team, or emergency services
- any person-centred dietary requirements
- any person-centred [cultural](#) requirements.

Monitoring blood glucose levels

Some people can do this for themselves. Where they do this, it must be:

- risk assessed
- reviewed at regular intervals
- reviewed if the person's condition changes.

Blood glucose levels will determine whether insulin doses need to change and by how much. Insulin doses may vary for people whose diabetes is difficult to control. Their blood glucose levels will determine how many units they take each time.

There are different types of blood glucose testing kits, which vary in how they are set up and used. Care home staff should have a process to ensure that these are calibrated and fit for purpose in line with the manufacturer's guidance.

Flash glucose monitoring

Some people monitor their blood glucose levels using a flash glucose monitor. This small sensor is worn just under the skin and records glucose levels continuously throughout the day and night.

Whenever necessary, people can scan the sensor to check their blood glucose level.

Staff should be trained and competent to use and support people to use a flash monitor.

Administering insulin

Self-administration

Some people will be able to administer their own insulin.

You need to carry out a risk assessment and record the risk to check that the person can do this safely. Although some people may be able to administer their insulin, they may need support with dialling the correct dose. You must record any support you give with medicines. Ensure the risk assessment includes person-centred medicines support to maximise a person's ability to self-administer. If a person is unable to manage any part of administering their insulin, nurses or trained care staff should administer.

Only trained and competent staff should be responsible for administering insulin or monitoring blood glucose levels. Where a person's diabetes care is stable, a care worker could complete these activities as a [delegated task](#) in accordance with national guidance.

Care staff must receive specialist training to administer insulin as a delegated task. They should be assessed as competent to administer insulin to the named person or people.

Find out more about [delegating medicines administration](#).

Records need to show how much insulin has been given on each occasion. District or community nurses may keep their own records. Care home staff must have a system of ensuring that information regarding insulin administered by district nurses or community nurses is available at the point of transfer of care.

Rotating injection sites

Staff need to rotate and record the injection site to avoid [lipohypertrophy](#). This is also known as lipos - hard lumps that can form if you inject the same place too often.

Common injection sites include upper arms, thighs, buttocks, and abdomen.

Accessories

A variety of hypodermic needles, syringes, lancets, and other accessories can help with administering and monitoring insulin. They are usually available on prescription.

Disposable insulin pens and insulin cartridges for re-usable pens should only be administered using the pen device with the appropriate needle.

Never use a syringe to extract insulin from pen devices or cartridges. This can lead to serious errors. Find out more in this [NHS Patient Safety Alert](#).

If using insulin from a vial, always measure insulin with an appropriate insulin syringe (marked in units/mL).

Disposal

Dispose of sharps in a suitable container and arrange to dispose of these containers appropriately. If visiting healthcare professionals are administering the insulin, ensure there is an agreement regarding the disposal of sharps.

Find out more about [handling sharps in adult social care](#).

Low blood glucose - hypoglycaemia

Insulin can cause low blood glucose levels (hypoglycaemia). The timing of insulin administration and meals is important. It helps to avoid fluctuations in blood glucose levels.

Be aware of the symptoms of low blood glucose levels. They can include:

- hunger
- anxiety or irritability
- palpitations
- sweating
- tingling lips.

If severe, it can lead to convulsions, loss of consciousness, coma and death.

A person-centred care plan should identify:

- the symptoms of a low blood glucose level
- the actions to take if a person's levels are low.

Some people might need to use emergency 'rescue' medicine or food to increase their glucose levels. For example, oral glucose or glucagon injection.

Care staff should have access to information about hypoglycaemia management.

High blood glucose - hyperglycaemia

There are several reasons why high blood glucose levels (hyperglycaemia) may happen. For example, if a person:

- has missed a dose of medicine
- has eaten more carbohydrate than the body or medicines (or both) can process
- is stressed
- is unwell from an infection
- has been over-treated for low blood glucose levels.

High blood glucose levels can cause [diabetic ketoacidosis \(DKA\)](#), which needs urgent treatment.

The person's care plan should specify:

- what blood sugar levels are appropriate for them
- the actions to take if their levels are high.

Managing complications of diabetes

People with diabetes can be more at risk of additional complications such as kidney, foot, and eye problems.

Care home staff should regularly examine the feet of people with diabetes and ensure there are measures to prevent pressure sores.

It is recommended that each person is registered with an optometrist and is offered regular eye tests.

Storing insulin

If not stored correctly, insulin products can lose their effectiveness. As insulin is a protein, it may break down if frozen or left out of the fridge for longer than the manufacturer specifies.

Medicines stored in a fridge should be between 2°C and 8°C.

Find out more about [storing medicines in a fridge](#).

Once the insulin is in use, it can usually be stored at room temperature for a limited period. Record the date when you first open or use an insulin product. This helps when checking how long the insulin has been out of the fridge. For detailed information, refer to the patient information leaflet for each product.

Labelling

Some multi packs of insulin pens are supplied in boxes labelled with the person's name. Sometimes the supplying pharmacist will label the individual pens. If individual pens are not labelled, make sure it is clear who the insulin pens belong to.

Accurate recording

When prescribing, transcribing, or recording insulin, do not abbreviate the word 'unit'. Always write it in full. Abbreviations (such as 'u') can be confused with a zero (particularly if handwritten). This could lead to serious harm.

This page is for:

- **adult social care services**
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Find out more

[Medicines: information for adult social care services](#)

For further advice, contact medicines.enquiries@cqc.org.uk

See also

[Diabetes UK](#)

[Delegating medicines administration](#)

[Storing medicines in fridges in care homes](#)

[NHS Patient safety alert: Risk of severe harm and death due to withdrawing insulin from pen devices](#)

[Handling sharps in adult social care](#)

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